

DISCHARGE SUMMARY

PATIENT DEMOGRAPHICS

Name: Patient 48

Age: 58 years old

Sex: Female

MRN: 000-TEST-48

Admission Date: [Admission Date]

Discharge Date: [Discharge Date]

Length of Stay: 3 days

PRIMARY DIAGNOSIS

Acute appendicitis with peritoneal involvement s/p laparoscopic appendectomy with intraoperative conversion to open approach

SECONDARY DIAGNOSES

- Systolic heart failure (EF 35-40%, NYHA Class III)
- Obstructive sleep apnea on CPAP therapy
- Hypertension
- Type 2 diabetes mellitus
- Hyperlipidemia

HOSPITAL COURSE

Patient 48 presented via ED with acute abdominal pain, fever (Temp 38.9°C), and elevated WBC (14,200/ μ L). Imaging (CT abdomen/pelvis) confirmed acute appendicitis with surrounding inflammatory changes concerning for perforation. Patient taken to OR for laparoscopic approach; however, due to severe adhesions and purulent fluid encountered, procedure was converted to open midline incision (approximately 6 cm) for adequate visualization and resection. Appendix removed en bloc with inflamed omentum. Peritoneal lavage performed.

Postoperatively, patient experienced expected pain (managed with IV hydromorphone initially, transitioned to oral analgesics POD#2). SpO2 monitored closely given sleep apnea history; CPAP restarted POD#1 at patient's baseline settings. Cardiac exam remained stable; BNP 620 pg/mL on admission, trending appropriately. Foley catheter removed POD#2; patient voiding without difficulty. Diet advanced as tolerated: clear liquids POD#1 → full liquids POD#2 → regular diet POD#3 with good tolerance. Trocar sites (3 small incisions at umbilicus, RLQ, and epigastrium) healing without erythema or drainage. No fever post-discharge planning.

DISCHARGE MEDICATIONS

| Medication | Dose | Frequency | Duration |

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| Acetaminophen | 650 mg | q.6h. PRN pain | 2 weeks |

| Oxycodone | 5–10 mg | q.4–6h. PRN moderate pain | 7 days only |

| Lisinopril (ACE inhibitor) | 20 mg | q.d. | Continue indefinitely |

| Furosemide (diuretic) | 40 mg | BID | Continue indefinitely |

| Metformin | 1000 mg | BID | Continue indefinitely |

| Atorvastatin | 40 mg | q.d. | Continue indefinitely |

| Aspirin | 81 mg | q.d. | Continue indefinitely |

ACTIVITY RESTRICTIONS & WOUND CARE

- No heavy lifting >10 lbs × 6 weeks
- No strenuous exercise or abdominal straining × 4 weeks
- Avoid driving while on oxycodone
- Keep midline incision and trocar sites clean and dry
- May shower once incision is fully sealed (typically 48 hrs post-discharge); pat dry gently
- Monitor incision for signs of infection (see RED FLAGS below)

DIETARY GUIDELINES

Resume regular diet as tolerated. High-fiber diet encouraged once fully healed to prevent constipation (risk with narcotic use). Fluid intake ≥ 2 L/day to maintain hydration.

FOLLOW-UP APPOINTMENTS

- General Surgery Clinic: in 2 weeks for incision check and suture removal
- Cardiology: within 1 week to reassess heart failure status post-surgery and adjust diuretics PRN
- Primary Care Physician: in 1